METROPOLITAN ACADEMIC MEDICAL CENTER

Department of Internal Medicine & Cardiology | 500 Hospital Plaza

DISCHARGE SUMMARY

Patient Name:	Robert Vance Miller	**MRN / ID:**	MRN-992014	**DOB / Age:**	11/14/1965 (60 yrs / M)
Admission Date:	10/24/2026	**Discharge Date:**	10/28/2026	**Room / Bed:**	402-B
Attending MD:	Dr. E. Vance, MD, FACC	**Referring MD:**	Dr. R. Sterling, MD	**Service:**	Cardiology

1. PRIMARY & SECONDARY DIAGNOSES

- **Primary Diagnosis:** Acute Coronary Syndrome – Non-ST-Elevation Myocardial Infarction (NSTEMI), successfully treated with percutaneous coronary intervention (PCI).
- **Secondary Diagnoses:** Essential Hypertension (Stage II), Type 2 Diabetes Mellitus (uncontrolled), Mixed Hyperlipidemia, mild chronic kidney disease (CKD Stage II).

2. CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS (HPI)

The patient is a 60-year-old male who presented to the Emergency Department via EMS complaining of severe substernal chest pressure radiating to the left jaw and shoulder. Symptoms began approximately 3 hours prior to presentation while resting at home. The pain was rated as 8/10 on the visual analog scale, accompanied by diaphoresis, shortness of breath, and mild nausea. Nitro sublingual administration by EMS provided partial symptomatic reduction to 5/10.

In the Emergency Department, initial electrocardiogram (ECG) demonstrated ST-segment depressions in leads V4–V6 and T-wave inversions in leads I and aVL without overt ST elevation. Serial High-Sensitivity Troponin I was significantly elevated at 3,450 ng/L (reference < 14 ng/L). Bedside echocardiogram revealed hypokinesis of the anterolateral wall with a preserved left ventricular ejection fraction (LVEF) of 50-55%.

3. ADMISSION PHYSICAL EXAMINATION

Vital Parameter	Admission Value	Reference Range	Status
Blood Pressure	168 / 94 mmHg	< 120/80 mmHg	**Elevated**
Heart Rate	92 bpm (Sinus)	60 – 100 bpm	Normal
Respiratory Rate	22 breaths/min	12 – 20 breaths/min	**Mild Tachypnea**
Oxygen Saturation (SpO2)	94% on Room Air	95 – 100%	**Borderline**
Temperature	36.8 °C (Afebril)	36.5 – 37.5 °C	Normal

Physical Exam Highlights: Regular rhythm, normal S1/S2, no S3/S4 or murmurs. Lungs clear to auscultation bilaterally without rales or wheezes. No peripheral edema (+0). Neurologically intact.

4. SUMMARY OF INPATIENT HOSPITAL COURSE

Coronary Angiography & Intervention: The patient was risk-stratified and taken urgently to the Cardiac Catheterization Laboratory. Diagnostic coronary angiography via right radial approach revealed 90% focal stenosis in the mid-Left Anterior Descending (LAD) coronary artery. The Left Circumflex (LCx) showed 30% luminal irregularity, and the Right Coronary Artery (RCA) was patent. Successful percutaneous coronary intervention (PCI) with a drug-eluting stent (DES) to the mid-LAD was performed with 0% residual stenosis and TIMI 3 flow restored.

Post-Procedure Management: The patient was monitored in the Cardiac Care Unit (CCU) for 24 hours. Dual Antiplatelet Therapy (DAPT) with Aspirin 81 mg daily and Ticagrelor 90 mg twice daily was initiated. High-intensity statin therapy (Atorvastatin 80 mg) and Beta-blocker therapy (Metoprolol Succinate 25 mg) were titrated without hemodynamic instability or bradycardia.

5. INPATIENT DIAGNOSTIC & LABORATORY FINDINGS

Laboratory Test	Admission Value	Day 2 Value	Reference Range
High-Sensitivity Troponin I	**3,450 ng/L**	**1,120 ng/L**	< 14 ng/L
Hemoglobin / Hematocrit	14.2 g/dL / 42%	13.8 g/dL / 41%	13.5–17.5 g/dL / 38-50%
Platelet Count	245 x10³/µL	210 x10³/µL	150 – 450 x10³/µL
Serum Creatinine	**1.35 mg/dL**	1.18 mg/dL	0.74 – 1.35 mg/dL
eGFR	**58 mL/min/1.73m²**	64 mL/min/1.73m²	> 60 mL/min/1.73m²
Fasting Blood Glucose	**184 mg/dL**	**142 mg/dL**	70 – 99 mg/dL
Hemoglobin A1c	**8.4 %**	--	< 5.7 %
LDL Cholesterol	**142 mg/dL**	--	< 70 mg/dL (High risk)

6. ECHOCARDIOGRAM & DIAGNOSTIC IMAGING REPORTS

Transthoracic Echocardiogram (Post-PCI): Left ventricular size is normal. Mild concentric left ventricular hypertrophy. Ejection fraction estimated at 52%. Regional wall motion abnormality noted in the anterior apex (mild hypokinesis). Right ventricle normal in size and function. No hemodynamically significant valvular lesions. No pericardial effusion.

7. DISCHARGE MEDICATION RECONCILIATION

Medication Name	Dosage & Route	Frequency	Indication / Special Instructions
Aspirin	81 mg PO	Once Daily	Antiplatelet therapy (Indefinite)
Ticagrelor (Brilinta)	90 mg PO	Twice Daily	DAPT post-PCI (Minimum 12 months)
Atorvastatin	80 mg PO	Once Daily at bedtime	High-intensity lipid management
Metoprolol Succinate	25 mg PO	Once Daily	Beta-blocker; hold if SBP < 100 or HR < 55
Lisnopril	5 mg PO	Once Daily	ACE Inhibitor for cardioprotection/BP
Metformin	500 mg PO	Twice Daily with meals	Glycemic control (re-started post-contrast)
Nitroglycerin Sublingual	0.4 mg SL	PRN Chest Pain	1 tab q5min x 3; Call 911 if pain persists

8. ACTIVITY, DIETARY & LIFESTYLE DIRECTIVES

- **Physical Activity:** No lifting heavy objects (> 10 lbs) for 2 weeks due to right radial access site healing. Avoid strenuous aerobic activity until cleared by outpatient cardiology. Daily light walking is encouraged.
- **Dietary Guidelines:** Strict adherence to a Heart-Healthy / Mediterranean Diet (< 2,000 mg Sodium per day, low saturated fats). Carbohydrate consistency for optimal diabetic control.
- **Wound Care:** Monitor right wrist catheter access site daily for signs of hematoma, active bleeding, severe bruising, or localized warmth/redness.

9. OUTPATIENT FOLLOW-UP SCHEDULE

Specialty / Provider	Timeframe	Location / Contact
Cardiology Outpatient Clinic	2 Weeks post-discharge	Suite 204, Heart Institute (Ext. 4100)
Primary Care Physician (PCP)	1 Week post-discharge	Main Street Medical Group
Endocrinology / Diabetes Educator	4 Weeks post-discharge	Outpatient Metabolic Center
Phase II Cardiac Rehabilitation	Enrollment within 3 weeks	Wellness Center Annex

CRITICAL WARNING SIGNS — WHEN TO SEEK IMMEDIATE MEDICAL ATTENTION:
Return to the nearest Emergency Department or Call 911 immediately if you experience any of the following symptoms:

- Recurrent chest discomfort, tightness, or pressure similar to your initial event.
- Sudden shortness of breath, severe dizziness, lightheadedness, or syncope (fainting).
- Active bleeding, rapid swelling, or a expanding firm knot (hematoma) at the wrist catheter site.
- Signs of infection including fever (> 38.0 °C / 100.4 °F), chills, or purulent drainage.

10. PATIENT EDUCATION & ACKNOWLEDGMENT SUMMARY

The patient was counseled extensively prior to discharge regarding diagnosis, drug action, side effect profiles (specifically bleeding risks associated with DAPT), lifestyle modifications, and emergency protocols. All questions were addressed, and verbal understanding was confirmed. Written instructions and drug schedules were provided in the patient's primary language.

11. CASE SUMMARY & PROGNOSTIC STATEMENT

The patient achieved successful revascularization following an uncomplicated NSTEMI. Cardiac biomarkers showed an appropriate trend toward normalization prior to discharge. Left ventricular function remains preserved. Long-term prognosis is favorable provided strict compliance with Dual Antiplatelet Therapy, lipid-lowering agents, glycemic control, and secondary prevention lifestyle adjustments is maintained.

12. PHYSICIAN ATTESTATION & SIGNATURES

I attest that I have personally evaluated, managed, and reviewed the clinical medical record for the patient during this admission. The discharge instructions and care summary accurately reflect the inpatient treatment provided and follow-up plan established.

Dr. E. Vance, MD, FACC
Attending Cardiologist
License #: MD-8829104

Dr. K. Miller, MD
Internal Medicine Resident
Hospital Staff Physician

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